SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

# Trastuzumab SC · early breast cancer

HER2-positive early breast cancer · SC component · NCCP 00285 · version 4 · 21-day cycle

| Patient name | DOB | Hospital no. | Date |
|---|---|---|---|
| | | | |

**Treatment context**
Trastuzumab under the skin is part of treatment to reduce recurrence in HER2-positive early breast cancer. The wider plan can include chemotherapy, surgery, radiotherapy and endocrine treatment when indicated.

**Schedule**
Trastuzumab 600 mg under the skin every 21 days, without a loading dose, to complete one year total. Previous IV or combination-treatment trastuzumab counts towards that year; a route switch does not start a new year.

## 1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Required blood tests and cardiac monitoring discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

## 2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

**Trastuzumab**
HER2-TARGETED ANTIBODY

- ☐ Heart pumping-function reduction / heart failure
- ☐ Allergic or administration-related reactions
- ☐ Injection-site pain, redness or swelling
- ☐ Diarrhoea, fatigue and rare serious lung effects

Discuss expected benefit, alternatives, route, duration, cardiac monitoring, pregnancy / contraception, urgent contacts and individual treatment risks. Add any companion drug using its own regimen information.

**Individual aim / benefit, alternatives, material risks and questions:**

## 3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

**Prototype only.** This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00285 v4.

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